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REF: QNI-PID-2070-1122-AL BETA TRIAL — ADVERSE OUTCOME PROJECTIONS | PRODUCT INTEGRATION DIVISION

DOCUMENT TYPE: Internal Projection Report — NOT FOR EXTERNAL DISTRIBUTION

PREPARED BY: Product Integration Division — Risk Assessment Unit

DATE: 22 November 2070

SUBJECT: Phase III Launch: Adverse Outcome Modeling — Final Projections

DISTRIBUTION: Executive Board Only — Level 7 Clearance Required

EXECUTIVE SUMMARY

This report presents final adverse outcome projections for the proposed Phase III consumer integration launch. All figures are derived from Phase I/II trial data (N=847,000) scaled to the proposed Phase III initial cohort of 340 million integration-eligible individuals across the Pacific Northwest, California, and British Columbia launch zones.

The Risk Assessment Unit presents three projection scenarios: Conservative, Baseline, and Accelerated. The Board has indicated a preference for the Baseline scenario for internal planning purposes. All three are presented for completeness.

SECTION A: NON-CRITICAL ADVERSE OUTCOMES

OUTCOME	CONSERVATIVE (0.8x baseline)	BASELINE	ACCELERATED (1.4x baseline)
Temporary cognitive disruption (>72hr, self-resolving)	680,000	850,000	1,190,000
Significant preference drift (documented, >20% baseline shift)	748,000	935,000	1,309,000
Persistent sensory artifact (earworm-class, chronic)	238,000	297,000	415,800
Impaired autonomous decision-making (6-12 months)	442,000	552,500	773,500
Impaired autonomous decision-making (>12 months)	204,000	255,000	357,000
Relationship/social functionality impairment	340,000	425,000	595,000

SECTION B: CRITICAL ADVERSE OUTCOMES

The following outcomes are classified as critical. All figures in this section have been reviewed and approved for inclusion in Board planning documents. Some line items have been redacted from standard distribution copies per Executive Board

instruction.

OUTCOME	CONSERVATIVE	BASELINE	ACCELERATED	STATUS
Permanent cognitive restructuring (irreversible)	250,800	374,000	523,600	DISCLOSED Ethics Cmte
Acute integration failure (hospitalization)	68,000	102,000	142,800	DISCLOSED Ethics Cmte
Substrate dependency (withdrawal-class)	510,000	765,000	1,071,000	DISCLOSED Ethics Cmte
Permanent personality dissociation	REDACTED	REDACTED	REDACTED	REDACTED Board Only
Mortality — direct causation	REDACTED	REDACTED	REDACTED	REDACTED Board Only
Mortality — indirect, 12-month post-integration	REDACTED	REDACTED	REDACTED	REDACTED Board Only

Redacted figures available to Board members with Level 9 clearance. Contact the Office of the Chief Risk Officer to request access (ref: [REDACTED]).

SECTION C: LAUNCH RECOMMENDATION

The Risk Assessment Unit presents the following analysis for Board consideration:

Projected integration cohort (Phase III, Year 1):	340,000,000
Projected beneficial outcomes (depression/anxiety reduction):	~289,000,000 (85%)
Projected non-critical adverse outcomes (total):	~3,314,500 (baseline scenario)
Projected critical adverse outcomes (disclosed):	~1,241,000 (baseline scenario)
Projected critical adverse outcomes (redacted):	[REDACTED]
Benefit-to-adverse-outcome ratio (disclosed figures only):	233:1

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RISK ASSESSMENT UNIT RECOMMENDATION: Based on disclosed adverse outcome projections, the benefit-to-risk ratio supports proceeding with Phase III launch on the proposed Q1 2071 timeline. The Unit notes that this recommendation is based on disclosed figures only. The Unit does not have visibility into the redacted outcome categories and cannot assess the complete risk profile. This limitation is noted for the record.